Materials Design Analysis Reporting (MDAR) Checklist for Authors

The MDAR framework establishes a minimum set of requirements in transparent reporting applicable to studies in the life sciences (see Statement of Task: doi:10.31222/osf.oio/9sm4x.). The MDAR checklist is a tool for authors, editors and others seeking to adopt the MDAR framework for transparent reporting in manuscripts and other outputs. Please refer to the MDAR Elaboration Document for additional context for the MDAR framework.

Materials

Antibodies	Yes (indicate where provided: page no/section/legend)	n/a
For commercial reagents, provide supplier name, catalogue number and RRID, if available	No antibodes detected. Please add identifiers for all resources where possible	

Cell Materials	Yes (indicate where provided: page no/section/legend)	n/a
Cell lines: Provide species information, strain. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No cell lines detected Please add identifiers for all resources where possible	
Primary cultures: Provide species, strain, sex of origin, genetic modification status.	Not currently checked by SciScore	

Experimental Animals	Yes (indicate where provided: page no/section/legend)	n/a
Laboratory animals: Provide species, strain, sex, age, genetic modification status. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No organisms detected Please add identifiers for all resources where possible	
Animal observed in or captured from the field: Provide species, sex and age where possible	Not currently checked by SciScore	
Model organisms: Provide Accession number in repository (where relevant) OR RRID	See laboratory animals section for information.	

Plants and microbes	Yes (indicate where provided: page no/section/legend)	n/a
Plants: provide species and strain, unique accession number if available, and source (including location for collected wild specimens)	Not currently checked by SciScore	
Microbes: provide species and strain, unique accession number if available, and source	Not currently checked by SciScore	

Human research participants	Yes (indicate where provided: page no/section/legend)	n/a
Identify authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	The study protocol (Supplement 1) was approved by an independent ethics committee or institutional review board at each study site (eTable in Supplement 2).	
Provide statement confirming informed consent obtained from study participants.	All patients provided written informed consent.	
Report on age and sex for all study participants.	**Age:**SHARPS enrolled adults (aged 18-65 years) diagnosed with HS 1 year or more prior to baseline. **Sex:**not detected.	

Design

Study protocol	Yes (indicate where provided: page no/section/legend)	n/a
For clinical trials, provide the trial registration number OR cite DOI in manuscript.	Not detected.	

Laboratory protocol	Yes (indicate where provided: page no/section/legend)	n/a
Provide DOI or other citation details if detailed step-by-step protocols are available.	Not detected.	

Experimental study design (statistics details)	Yes (indicate where provided: page no/section/legend)	n/a
State whether and how the following have been done, or if they were not carried out		
Sample size determination	Based on the combined response rates for the HiSCR from previous studies, 9 a conservative estimate of 20% treatment difference was used for the power calculation in which case 100 patients per treatment arm would provide at least 80% power to detect a treatment difference at the alpha level of .05 and to demonstrate the point estimate of the treatment difference of at least 15% for the primary end point.	
Randomization	Methods Study Design The SHARPS study was a global, phase 4, prospective, multicenter, randomized, double-blind, placebo-controlled study of adalimumab in conjunction with surgery in adults with moderate to severe HS who were surgical candidates.	
Blinding	not detected.	
inclusion/exclusion criteria	Patients with a baseline draining fistula count of more than 20; those requiring surgical management before week 13; those requiring excisional surgery with primary closure, partial surgical reduction of excision with surgical suture, or reconstruction techniques; and those with active skin disease or conditions other than HS that could interfere with the assessment of HS were excluded.	

Sample definition and in-laboratory replication	Yes (indicate where provided: page no/section/legend)	n/a
State number of times the experiment was replicated in laboratory	Not detected.	
Define whether data describe technical or biological replicates	Not detected.	

Ethics	Yes (indicate where provided: page no/section/legend)	n/a
Studies involving human participants: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	The study protocol (Supplement 1) was approved by an independent ethics committee or institutional review board at each study site (eTable in Supplement 2).	
Studies involving experimental animals: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	Not detected.	

Ethics	Yes (indicate where provided: page no/section/legend)	n/a
Studies involving specimen and field samples: State if relevant permits obtained, provide details of authority approving study; if none were required, explain why.	Not detected.	

Dual Use Research of Concern (DURC)	Yes (indicate where provided: page no/section/legend)	n/a
If study is subject to dual use research of concern, state the authority granting approval and reference number for the regulatory approval	Not currently checked by SciScore	

Analysis

Attrition	Yes (indicate where provided: page no/section/legend)	n/a
State if sample or data point from the analysis is excluded, and whether the criteria for exclusion were determined and specified in advance.	During the perioperative period (weeks 13 and 14), patients continued adalimumab, 40 mg, weekly or placebo, and the surgeon measured and recorded the surface area of the actual surgery.	

Statistics	Yes (indicate where provided: page no/section/legend)	n/a
Describe statistical tests used and justify choice of tests.	Categorical variables were analyzed using Cochran-Mantel-Haenszel test, stratified by baseline Hurley stage and anatomical location of the HS surgical site and continuous variables using analysis of covariance, with treatment, baseline Hurley stage, anatomical location of the HS surgical site, and baseline values in the model.	

Data availability	Yes (indicate where provided: page no/section/legend)	n/a
State whether newly created datasets are available, including protocols for access or restriction on access.	Not detected.	
If data are publicly available, provide accession number in repository or DOI or URL.	Not detected.	
If publicly available data are reused, provide accession number in repository or DOI or URL, where possible.	Not detected.	

Code availability	Yes (indicate where provided: page no/section/legend)	n/a
For all newly generated code and software essential for replicating the main findings of the study:		
State whether the code or software is available.	Not detected.	
If code is publicly available, provide accession number in repository, or DOI or URL.	Not detected.	

Analysis

Adherence to community standards	Yes (indicate where provided: page no/section/legend)	n/a
MDAR framework recommends adoption of discipline-specific guidelines, established and endorsed through community initiatives. Journals have their own policy about requiring specific guidelines and recommendations to complement MDAR.		
State if relevant guidelines (eg., ICMJE, MIBBI, ARRIVE) have been followed, and whether a checklist (eg., CONSORT, PRISMA, ARRIVE) is provided with the manuscript.	Not currently checked by SciScore	